NIHR | National Institute for Health and Care Research

NIHR Fellowships Programme

Funding, Support or Development Opportunity	2026/343 Predoctoral Award (Cohort 2)

SUMMARY INFORMATION

Contracting Organisation	GREAT ORMOND STREET HOSPITAL FOR CHILDREN NHS FOUNDATION TRUST
Start Date	01/10/2026
End Date	30/09/2027
Duration (months)	12
% FTE Commitment	100%
Percentage of award time undertaking practice/clinical work	0%
Total Research Costs Requested	£108,680.70
Has this application been previously submitted to this or any other funding body?	No
Partnership Fellowship	Not Applicable

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LEAD APPLICANT & RESEARCH TEAM

Lead Applicant

Full Name	Ms Katya Herman
Organisation	GREAT ORMOND STREET HOSPITAL FOR CHILDREN NHS FOUNDATION TRUST
Department	Therapies
Professional Background	Allied Health Professional
Detailed Professional Background	Music therapist
Professional Registration	HCPC Arts Therapist: AS15858
ORCID	0009-0006-4537-7161
Research Degree Status	Registered
Contextual Factors	n/a

APPLICATION DETAILS

ODA Compliance
n/a

Working with People and Communities Summary
Music therapy is collaborative by nature and, in paediatric care, must be shaped entirely by what matters to children, young people and families. Music therapy research should be no different. Community Engagement and Involvement (CEI) has been part of my research from the beginning: during my GOSH Biomedical Research Centre (BRC) internship, I spoke with six families from diverse backgrounds (Bangladesh, Ghana, Romania, Ireland, UK) about my research plan. Short conversations at convenient times, with plain-English summaries provided, demonstrated respect for families' time. Families felt the research was worthwhile and expressed interest in being involved in my project. Four carers highlighted that fixed-time meetings could be difficult due to the unpredictability of inpatient care; two noted the challenge of balancing hospital attendance with childcare. Two families for whom English was not a first language suggested receiving translated information and questions in advance. These early discussions have already shaped how I will approach my research during both my predoc award and doctoral studies. Their reflections highlighted the need for research processes that fit around the realities of inpatient life: families' emphasis on unpredictability, competing caregiving demands and emotional fatigue reinforced my commitment to designing research that is flexible, responsive and paced by participants' capacity at any given time. Feedback from families for whom English was not a first language reinforced the importance of prioritising clear, accessible communication, including translated materials and advance sharing of questions, to enable meaningful engagement.

I will work with 8-10 parent or carer contributors and, where appropriate, 3-5 young people with experience of inpatient music therapy. Recruitment will take place in collaboration with clinical teams, using sensitive and timely approaches, either in person or via brief phone calls. Recognising the time and emotional pressures on families in inpatient settings, their early involvement will be flexible and informal conversations, exploring priorities and experiences, including what mattered during hospital stays, challenges within music therapy, and meaningful outcomes. Emerging findings will be shared in accessible ways, with invitations for feedback ("Here's what I'm finding so far - does this reflect your experience?"). This will help capture nuance and context that might otherwise be overlooked.

To ensure my research remains grounded in real-world priorities, CEI will be integrated across key stages: project set-up (refining research questions); data analysis (sense-checking emerging themes, validating findings, and identifying gaps or alternative perspectives); and dissemination (co-creating accessible summaries and presentations; deciding how results are shared).

I will aim to ensure contributors reflect, as far as possible, the diversity of families accessing inpatient services. I will work with GOSH PPIE leads to support inclusive involvement and reduce barriers to participation, including the use of interpreters or translated materials where required, and adapting methods for families with disabilities or additional needs. Contributors will be paid for their time.

By involving people with lived experience at every stage, I aim to produce research that is relevant, respectful and genuinely informed by those it is intended to benefit.

SUPPORT AND MENTORSHIP

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Full Name	Ms Kirsty Jane
Organisation	King's College London
Email Address	kirsty.jane1@nhs.net
Role	Mentor
Description of Support Provided	Kirsty Jane is a specialist music therapist with expertise in neonatal palliative care. She is a Doctoral Clinical Academic Fellow at King's College London and was awarded an NIHR-funded Doctoral Clinical Academic Fellowship in 2023. She has extensive experience in collaborative methods and PPIE, working closely with people with lived experience to increase the accessibility of psychotherapeutic support and dissemination of findings. Her work focuses on improving the acceptability and accessibility of music therapy for parents, carers, and healthcare professionals during uncertainty. Ms Jane will serve as an excellent sounding board for Katya on the clinical realities and therapeutic nuances of music therapy, ensuring the realist review remains clinically meaningful. She will also provide invaluable support for Katya's professional development as a clinical academic, including guidance in preparing for future doctoral studies and effective patient and public engagement and involvement. Katya will meet with Ms Jane monthly to discuss the development of PPIE that will inform future doctoral fellowship applications, creative and accessible methods for dissemination of findings as well as reviewing the clinical relevance and implications of the findings for practice.

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Full Name	Ms Jasmine Edwards
Organisation	New York University (USA)
Email Address	Jne254@nyu.edu
Role	Mentor
Description of Support Provided	Jasmine Edwards is a practising Music Therapist and Doctoral Candidate and Fellow at New York University, with a focus on cultural humility, inclusive practice, and the lived experiences of under-represented communities. She has served as adjunct faculty in Music Therapy departments at Howard University, NYU, Montclair State University, Nazareth College, Duquesne University, and Molly University. Ms Edwards will provide mentorship to Katya in developing culturally responsive research questions that are driven by the experiences and voices of the children and families served. She will support the sensitive interpretation of diverse patient and family perspectives, and the embedding of equity and inclusion throughout the review methodology, aligning the project with NHS priorities on inclusive practice. Katya will meet with Ms Edwards quarterly to ensure these principles are consistently applied throughout the project. Looking forwards, and in addition to project-specific mentorship, Ms Edwards will support Katya Herman's development throughout her progression toward a clinical academic career. Drawing on her experience as both a practising clinician in paediatric hospitals, and doctoral fellow, Ms Edwards will provide guidance on balancing clinical practice with academic responsibilities, developing a sustainable research identity, and navigating the expectations of doctoral study. Through regular mentoring discussions, Ms Edwards will support Katya in building confidence as an independent researcher, strengthening interdisciplinary collaboration skills, and positioning her work to contribute meaningfully to both clinical practice and academic scholarship.

Full Name	Professor Jo Wray
Organisation	GREAT ORMOND STREET HOSPITAL FOR CHILDREN NHS FOUNDATION TRUST
Email Address	jo.wray@gosh.nhs.uk
Role	Primary Academic Supervisor

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Description of Support Provided	Professor Jo Wray is an experienced health psychologist at Great Ormond Street Children's Hospital, London and Professor of Child Health Psychology – Cardiology, Critical Care and Transplantation in the Institute of Cardiovascular Science at UCL. She leads a programme of psychosocial research at GOSH with a particular focus on the psychosocial impact of chronic and rare health conditions in children and patient-reported experience and outcomes. She is a senior research fellow in the Centre for Outcomes and Experience Research in Children's Health, Illness and Disability (ORCHID) at GOSH. Across her GOSH and UCL roles she supports nursing and allied health research at all levels. She has supervised students at all stages of the clinical academic pathway including nine PhD students (three of whom have been NIHR funded) and four clinical psychology doctoral trainees to successful completion, together with eight Master's students. She is currently supervising nine doctoral students, five of whom will complete in 2026/7. As Professor Wray works in the same institution as the applicant, provision of regular supervision will be easily accommodated, and she has protected time to provide academic supervision. Professor Wray is a mixed methods researcher and brings methodological expertise to the current predoctoral application and planned doctoral application. She has considerable experience of working with children, families and staff using a wide range of methodologies, including focus groups, on-line forums, interviews and questionnaires (including questionnaire development) and has also undertaken a number of scoping and systematic reviews. She has published over 200 peer-reviewed articles and contributed to a number of international guidelines. Professor Wray is currently leading or a co-applicant on several national and international funded studies which include qualitative approaches to exploring patient, family and staff experiences of illness and transition to adult services, quantitative elements related to clinical and psychosocial data, systematic, scoping, and realist reviews and evidence synthesis. Professor Wray and Katya have previously collaborated: Professor Wray provided guidance to create and develop the Patient- and Family-Reported Experience Measures used by the Music Therapy service at GOSH. This established working relationship demonstrates Professor Wray's knowledge of Katya's capabilities and provides a strong foundation for continued mentorship. For this project, Professor Wray will provide academic leadership and methodological supervision, supporting Katya in realist review methodology, critical appraisal, qualitative synthesis, and academic writing. Professor Wray will meet monthly with Katya to review progress, discuss emerging findings, and provide structured mentorship, including guidance on publication strategy, and engagement with research networks. She will also ensure the review is contextualised within the psychosocial and clinical challenges faced by hospitalised children and families, strengthening the translational potential of the findings. Professor Wray will ensure Katya's academic development needs are met during the NIHR Pre-Doctoral Fellowship and she will continue to support the development of Katya's research ideas and advancement of her future academic career as she prepares her doctoral fellowship application.

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Full Name	Professor Dr Annie Heiderscheit
Organisation	Anglia Ruskin University
Email Address	Annie.Heiderscheit@aru.ac.uk
Role	Academic Supervisor

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Description of Support Provided	Professor Annie Heiderscheit is Professor of Music Therapy and Director of the Cambridge Institute for Music Therapy Research at Anglia Ruskin University. She is a board-certified music therapist (MT-BC) with over 32 years of advanced clinical practice and more than 25 years' experience as a music therapy educator and researcher. Professor Heiderscheit has an outstanding international reputation for leading interdisciplinary research programmes and has methodological expertise in evidence synthesis, systematic reviews, and the evaluation of complex music-based interventions across paediatric, medical, critical care, transplantation, and mental health contexts. Her research is firmly grounded in clinical practice and focuses on improving patient and family outcomes during hospitalisation, including organ and bone marrow transplantation and other high-acuity care pathways. She has made a substantial and sustained contribution to the field, authoring over 50 peer-reviewed journal articles, more than 35 book chapters, and five authoritative textbooks on music therapy practice and research. This body of work demonstrates her leadership in advancing rigorous, clinically relevant evidence to inform healthcare practice and policy. Professor Heiderscheit has extensive experience supervising postgraduate research students, having successfully supervised three PhD students and approximately forty master's students to completion. She is currently supervising two PhD students and eleven master's students across early-, mid-, and late-stage study, demonstrating sustained supervisory capacity and experience supporting trainees at different stages of academic development. Professor Heiderscheit's expertise aligns directly with the aims and methodology of Katya's proposed fellowship, particularly in relation to paediatric music therapy within medical settings and the robust evaluation of psychosocial and experiential outcomes. She will provide senior methodological mentorship on the theoretical and practical foundations of a realist review, including protocol development, critical appraisal, synthesis of qualitative and quantitative evidence, and transparent reporting in line with best practice standards. This will ensure the review achieves a high level of disciplinary rigour and relevance to clinical care. In addition, Professor Heiderscheit will support Katya's development as an emerging clinical academic by fostering specialist expertise in paediatric music therapy and guiding the translation of evidence into clinically meaningful and implementation-focused insights. She will meet with Katya monthly to provide structured supervision, including progress review, methodological troubleshooting, interpretation of findings, and strategic career development advice. Professor Heiderscheit brings a combination of extensive clinical experience internationally recognised methodological expertise, and proven leadership in music therapy research, placing her in an exceptional position to support Katya in successfully delivering the fellowship objectives. Her mentorship will play a critical role in equipping Katya with the skills, confidence, and academic foundation required to progress towards a sustainable career as a clinical academic aligned with NIHR priorities.

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SUMMARY BUDGET

Justification of Costs
The requested funding will provide essential support for my development as a clinical academic, enabling me to undertake research training, attend relevant conferences, and engage in patient and public involvement activities. This will build the skills, knowledge, and networks required to prepare a competitive Doctoral Award application. Each cost has been carefully considered to ensure value for money, directly supports the proposed work, and aligns with the NIHR's objectives of developing high-quality, patient-centred research.

Salary Costs (£88,060.70)
Funding for my salary will provide protected time to undertake comprehensive research training, develop academic skills, and engage meaningfully with patients and communities. Covering my salary through this award will allow me to fully commit to my Master's in Research (MRes) training, acquire both practical and theoretical training on design and conduct of a research project, conduct my realist review, and prepare a competitive Doctoral Award application. This support is essential to build the foundation for a sustainable career as a clinical academic.

Training
UCL Child Health MRes Fees (£16,800)
This component of the award is critical given my limited prior research experience. The Child Health MRes will provide advanced training in research methodology, statistics, and study design specifically tailored to paediatric health research. The course at UCL was chosen carefully for its relevance to my clinical background and its focus on training researchers to conduct meaningful, patient-centred studies. Modules also cover working with children and families, research ethics, and dissemination of findings. Completing the MRes will equip me with the skills necessary to design and deliver a robust research project and prepare a competitive Doctoral Award application.

Cultural Competence Training at the Centre for Ethnic Health Research (£500)
This training covers inclusive, accessible, and culturally sensitive approaches to CEI, ensuring that lived-experience perspectives are meaningfully incorporated into research design, interpretation, and dissemination. By enhancing my understanding of the cultural and social factors that influence engagement, it will improve the quality, relevance, and impact of the research while supporting ethical, patient-centred practice.

Conference Attendance (£1,000)
I plan to attend two conferences directly relevant to my proposed research in paediatric music therapy: the 12th Nordic Music Therapy Conference, and the British Association for Music Therapy Conference 2026. Attendance at these conferences will support dissemination of early work, provide feedback from experts in the field, and contribute to the development of a competitive future Doctoral Award application. The total requested amount of £1,000 is broken down as follows:
Nordic Music Therapy Conference (£750 total)
Registration: £350
Travel: £250 (conference is in Iceland)
Accommodation (2-3 nights, budget rate): £150

British Association for Music Therapy Conference (£250 total)
Registration: £250

These conferences will enable me to present preliminary ideas, engage with current methodological developments in music therapy research, and build professional networks with clinicians and researchers in my field. Funding will ensure full participation, strengthening my academic development and future research applications.

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Community Engagement and Involvement (£1,320)
Funding is requested to compensate families for their time in four short, informal meetings to provide feedback and guidance on the research and dissemination. I anticipate approximately 6-8 families participating, with a possible 2 family members per family, giving around 12 participants in total. Each session will last approximately one hour with little or no preparation, in line with NIHR guidance (£27.50 per hour). This ensures fair reimbursement for family involvement while acknowledging the realities of inpatient family schedules.

Reimbursement acknowledges participants' time and expertise, supporting meaningful involvement in shaping research questions, interpreting findings, and co-producing dissemination outputs. This approach ensures the research is patient-centred, ethically robust, and relevant to families' real-world priorities.

Supervision and Mentoring (£1,000)
This covers the time of two experienced supervisors and two mentors, providing project oversight, methodological guidance, and professional support. Supervision ensures high-quality research conduct, adherence to ethical standards, and guidance for my career development as a clinical academic.

Overall the requested funding provides comprehensive support for my development as a clinical academic, ensuring protected time, relevant training, mentorship, and engagement with both the research community and families.

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ACKNOWLEDGEMENT AND CONFLICTS

I have read and understood the terms of applying as Lead Applicant and accept this role	✓

Potential Conflicts	n/a

Lead Applicant:	Katya Herman	**Application Number:**	NIHR515958
Contracting Organisation:	GREAT ORMOND STREET HOSPITAL FOR CHILDREN NHS FOUNDATION TRUST		
Proposed Start Date:	01/10/2026	**End Date:**	30/09/2027

Contributions

Katya Herman

Module 1 – Contributions to the generation of new ideas, tools, methodologies or knowledge

In 2019, I set up the first music therapy service at Great Ormond Street Hospital, establishing a new clinical model within a tertiary paediatric NHS setting. This involved designing referral pathways, governance processes, and outcome measurement systems integrated into existing hospital infrastructure. To date, the service has delivered over 6,500 sessions to more than 1,200 children across 24 wards, supporting patients with complex medical, developmental, and psychosocial needs. In 2020, the Care Quality Commission cited music therapy at GOSH as an example of Outstanding Practice for improving patient and family experience.

I also designed an embedded mixed-methods service evaluation framework, later adopted by GOSH's Play and Learning Disability teams to support learning from routine care. In collaboration with the GOSH Clinical Outcomes team, I led an evaluation integrating routine activity data, qualitative feedback from patients, families, and staff, and patient- and carer-reported outcomes. Findings showed that music therapy supported children's emotional wellbeing, self-expression, engagement, and coping, while also reducing stress and improving mood for parents and carers. These insights informed service refinements, revised referral criteria, and a successful business case for permanent staffing. Findings were shared with senior Trust leadership and at the GOSH Conference. This service evaluation's outcomes also highlighted unanswered questions around mechanisms of action, developmental outcomes, and equitable access, directly informing my proposed pre-doctoral research focus.

During the COVID-19 pandemic, I rapidly transitioned provision to a digital model, delivering over 225 remote sessions and developing online training for psychosocial staff to adapt care delivery. This experience revealed questions around acceptability, feasibility, digital exclusion, and clinical effectiveness, strengthening my interest in evaluating innovative service models and health equity. Collectively, these experiences demonstrate my capacity to develop and evaluate complex interventions within NHS contexts, providing a strong foundation for pre-doctoral research training.

Module 2 – Contributions to the development of others and maintenance of effective working relationships

I have extensive experience in the education and supervision of students, early-career clinicians, and multidisciplinary colleagues. At Massachusetts General Hospital (USA), I supervised seven undergraduate music therapy students across placements and internships in high-acuity clinical environments. In 2020, I established the first Music Therapy student placement programme at GOSH, designing a structured syllabus aligned with HCPC competencies and service priorities. To date, I have supervised nine postgraduate students, five of whom have progressed into NHS roles and service development posts. I have also contributed to education in specialist and community contexts. At Stephen Hawking School, a specialist school for children with profound and multiple learning disabilities, I delivered staff training on communication, sensory engagement, and inclusive therapeutic approaches. This experience strengthened my facilitation skills and supported the development of multidisciplinary staff working with highly complex populations. Within GOSH, I contribute to workforce development through teaching, ward-based education, and reflective practice with Allied Health Professionals and medical teams. Externally, I deliver validated education to healthcare professionals across NHS, hospice, and care-home settings, including postgraduate-level and specialist short courses in paediatric music therapy, mental health, and non-pharmacological symptom management in palliative care.

I maintain effective working relationships through close collaboration with multidisciplinary colleagues and by actively integrating diverse professional expertise into clinical practice. Recognising the need for more coordinated therapeutic input for children with prolonged admissions, I established monthly multidisciplinary meetings on the Respiratory and Cardiac wards to jointly review developmental and psychosocial goals. These forums enabled shared decision-making and improved communication between professions, leading to the introduction of two weekly therapy groups that provide coordinated multidisciplinary input and make more effective use of therapy resources for long-term patients.

Module 3 – Contributions to the wider research community

Prior to joining GOSH, I co-designed and delivered a 9-month music therapy pilot on Neurology and Trauma wards at the Royal London Hospital, presenting the outcomes at national multidisciplinary conferences, and contributing to the evidence base for music therapy within neurorehabilitation and trauma care. I also worked as a Research Assistant on a 3-year study of music therapy for children and young people with Batten disease, coordinating multi-site data

collection, conducting quantitative and thematic analysis, and contributing to the final integrated mixed-methods report. Findings were disseminated at national and international conferences and shared at family support network events, informing interdisciplinary practice and highlighting gaps in non-pharmacological research for rare neurodegenerative conditions. I serve as music therapy representative on the James Lind Priority Setting Partnership Steering Committee for Allied Health Professionals, contributing clinical expertise to national research prioritisation. I also co-lead a national Special Interest Group for music therapists in paediatric medical settings (though the British Association for Music Therapy) facilitating collaboration and dissemination of practice-based evidence. Through these activities, I support research prioritisation, collaboration, and the dissemination of practice-based evidence across clinical, academic, and patient communities, strengthening the evidence base for allied health interventions in complex paediatric settings.

Module 4 – Contributions to broader society

As Head of Music Therapy at GOSH, my work is underpinned by a commitment to culturally responsive and accessible care. In practice, this involves co-producing sessions in real time with children and families at the bedside, adapting music, communication, and session structure to their developmental and clinical needs. Through this approach, I aim to provide accessible psychological support that recognises and affirms each child's identity, cultural values, and lived experience within healthcare. This is particularly important for children with learning disabilities and complex communication needs, who may be excluded from verbally mediated therapies. Continuous patient and family feedback informs service development, and I remain attentive to barriers faced by families with language, cultural, or socioeconomic challenges, which has strengthened my commitment to inclusive communication and equitable involvement in care. These experiences also inform my developing research interests in designing approaches that better reflect the diversity of families accessing paediatric inpatient services.

Beyond clinical practice, I contribute to public and professional understanding of music therapy and psychosocial care through teaching, workshops, and careers outreach (e.g. NHS Kickstart), supporting widening participation in allied health professions, particularly among students from underrepresented backgrounds. Within healthcare, I mentor junior therapists and provide clinical supervision, supporting colleagues to develop reflective and inclusive practice, strengthening

workforce capability in paediatric psychosocial care. Beyond healthcare, I have contributed to community organisations supporting young people living with long-term and life-limiting conditions. At Next Step, I ran workshops for young people living with HIV/AIDS, cancer, and rare and genetic conditions to develop leadership and advocacy skills, supporting participants to create action plans to pursue goals that mattered to them. I also volunteered with The Literacy Pirates, supporting children to build literacy, self-belief, and perseverance through creative work.

My daily work focuses on improving access to psychosocial support for children and families with complex and diverse needs. Going forwards, I seek to contribute to a healthcare culture that recognises psychosocial care as essential rather than optional, and that meaningfully incorporates the voices of children, young people, and their families.

Module 5 – Your suitability for this award

My career reflects sustained commitment to combining clinical leadership with evidence-based practice. I hold a strong academic record (University of Cambridge, Ted Hughes Scholar), awards for clinical and academic excellence at Berklee College of Music, and advanced certifications (Neurologic Music Therapist, NICU Music Therapist, MATADOC Assessor). Alongside international clinical training, these experiences have strengthened my cross-cultural perspective and shaped my commitment to inclusive, developmentally appropriate psychosocial care. To progress to doctoral-level research, I now require structured training in outcomes evaluation, study design, and applied health research methods. Support from the NIHR Predoctoral Award would provide the methodological training, supervision, and protected time needed to prepare a competitive doctoral fellowship application and develop clinically grounded research with direct relevance to NHS service delivery and policy. My aim is to establish a clinical academic career that generates robust evidence to inform psychosocial care in paediatrics and improve equitable access to therapeutic support for children and young people with complex health needs.

Research Area of Interest

My research aims to understand the impact of music therapy for children and young people in paediatric inpatient settings, specifically how, why, and under what circumstances it produces positive outcomes.

Paediatric admissions present distinct challenges, including medical procedures, sterile and unfamiliar environments, heightened stress and anxiety, and disruptions to home life and daily routines. Music therapy is increasingly recognised as a valuable intervention in paediatric healthcare, with evidence suggesting benefits for anxiety, mood, coping, and overall wellbeing. Drawing on over ten years' experience working in hospital settings, I have observed how tailored music therapy can offer comfort, agency, and opportunities for connection and self-expression, supporting children's wellbeing within complex clinical environments. Emerging evidence, including findings from my previous service evaluation, also suggests that music therapy may simultaneously benefit parents and carers, supporting emotional regulation, reducing stress, and enhancing the shared experience of hospitalisation. This highlights the potential of music therapy to address multiple needs within a single intervention.

Despite these observed benefits, music therapy is a complex intervention, and the mechanisms through which it works, along with the contextual factors influencing its effectiveness across inpatient settings, remain under-explored. Outcomes are shaped not only by the intervention itself, but by interacting factors such as the child's developmental stage and diagnosis, family involvement, ward context, other therapeutic input, and the therapist's approach. These insights have motivated me to examine how music therapy produces meaningful outcomes in real-world NHS contexts, with the aim of informing evidence-based practice, supporting equitable delivery, and improving integration within paediatric care pathways.

To address this complexity, I propose a focused realist-informed review of existing evidence (a realist review conducted at master's level with pragmatic constraints) restricted to paediatric inpatient settings and psychosocial outcomes. A realist approach is particularly appropriate as it moves beyond asking whether music therapy works to examining context-mechanism-outcome configurations. With supervision from researchers experienced in realist

methodology and complex healthcare interventions, I will identify how specific contextual features (e.g. acute ward environment, family presence, developmental needs) interact with underlying mechanisms to produce different outcomes across groups and settings.

This work will generate patient-centred, actionable findings and a refined programme theory to guide future research. It will provide a strong conceptual and methodological foundation for doctoral study, bridging my clinical leadership with a PhD focused on developing and evaluating an equitable, clinically embedded music therapy framework to inform prioritisation and delivery of limited resources across paediatric inpatient services within the NHS.

Training

A central component of my career development is the UCL Child Health MRes, which I intend to pursue full-time. This programme will provide advanced, structured training in research methodology, statistics, and study design, equipping me with the skills required to conduct high-quality paediatric research. The core modules - Applied Statistics for Health Research I and a year-long independent research project tailored to my interests - will give me hands-on experience in designing and executing research studies, analysing complex data, and critically appraising evidence.

I plan to select three additional modules: Practising Qualitative Research in Child and Adolescent Health, International Child Mental Health, and Critical Thinking and Science Communication. These choices align directly with my experience as a music therapist working with children and families, providing opportunities to integrate clinical insight with rigorous research methodology. They will allow me to explore the psychological, social, and developmental dimensions of paediatric care, strengthen my capacity to design meaningful studies, and develop skills in translating findings into practice.

This combination of modules and project work will enable me to undertake a realist review in paediatric music therapy, synthesising complex evidence to address questions of intervention effectiveness, context, and mechanism. I will also benefit from access to ORCHID at GOSH, a leading centre for child health research methodology: through ORCHID's AIM Group (Applied and Implementation Methodology), I can receive in-house methodological training, including a targeted two-hour session on realist review methods. This focused support will directly inform the design, conduct, and rigour of my review, enhancing my knowledge of evidence synthesis, critical appraisal, and theory-driven evaluation. By systematically examining how and why music therapy works for children in different settings, and under what conditions, I will develop robust methodological skills and a solid foundation for a competitive Doctoral Award application.

In addition, Cultural Competence Training at the Centre for Ethnic Health Research will enhance my ability to design inclusive, culturally responsive research, ensuring that my work engages equitably with families from diverse backgrounds and produces outcomes relevant to the communities I serve.

Taken together, this training represents a critical bridge between clinical practice and academic research, enabling me to transition from delivering music therapy interventions to generating new evidence that informs policy, service delivery, and professional practice.

Building Collaborations

Collaboration will be central to my development as a clinical academic, ensuring my research is relevant, rigorous, and impactful. I will work with:

- Supervisors and mentors, who will provide expert guidance on realist review methodology, research design, and project oversight, while supporting the preparation of a Doctoral Award application. Their mentorship will be pivotal in shaping my transition from practitioner to independent researcher.
- Patient and public contributors, via structured PPIE sessions, to refine research questions, interpret findings, and co-produce outputs. This involvement will ensure that my research remains grounded in the lived experience of children and families, a principle central to both music therapy and child health research.
- Clinical teams at GOSH, who will provide insight into practical priorities, contextual constraints, and service delivery realities. Collaborating with clinicians will enable me to translate research into actionable improvements in care, reinforcing the link between academic inquiry and clinical impact.

Through these collaborations, I will not only strengthen my academic profile but also build the professional networks, mentorship relationships, and interdisciplinary perspectives essential for becoming an independent clinical academic.

Conferences

Dissemination is a core component of my development as a clinical academic. Attendance and presentations at professional conferences will enable me to share findings, gain critical feedback, and build visibility within both the music therapy and academic research communities.

- 12th Nordic Music Therapy Conference: I will present preliminary findings from my realist review, engage with international experts, and discuss methodological approaches and outcomes. This forum will provide an opportunity to position my work within a global context and receive feedback to refine subsequent research.
- British Association for Music Therapy 2026 Conference: Disseminating findings will enable engagement with clinical peers, stimulate discussion around implementation, and highlight the contribution of rigorous research to advancing practice.

By combining rigorous research training, interdisciplinary collaboration, and strategic dissemination, I will develop the skills, networks, and profile required to transition from music therapist to clinical academic, demonstrating the capacity to generate

evidence that directly informs child health practice and policy, and laying the foundation for a successful Doctoral Award application.

Great Ormond Street Hospital for Children
NHS Foundation Trust

Letter of Support from Libby Hough, Head of Therapies
Great Ormond Street Hospital for Children NHS Foundation Trust

I am pleased to provide my full support for Katya Herman's application to the NIHR Pre-Doctoral Clinical Academic Fellowship. As Head of Therapies at Great Ormond Street Hospital, I strongly endorse this application and confirm our organisational commitment to supporting Katya Herman in developing as a future clinical academic leader in paediatric music therapy.

Katya will be clinically hosted by Great Ormond Street Hospital for Children NHS Foundation Trust, with UCL as her academic organisation. GOSH works in close strategic partnership with University College London Great Ormond Street Institute of Child Health (UCL GOS ICH). Together, these organisations form the largest concentration of paediatric health research in Europe, with over 500 active research projects at any one time. GOSH is an NIHR-designated Intelligent Research Hospital, where research is embedded into clinical care to improve outcomes, experience, and quality of life for children and young people with complex and rare conditions.

GOSH has a strong track record in supporting clinical academic careers, including those of Allied Health Professionals. Our five-year organisational strategy, *Together We Power Care*, prioritises research as a core function of the Trust and commits to developing staff capability and capacity in research alongside clinical excellence. A central aim of this strategy is to involve staff from all professional groups in research activity to accelerate the translation of evidence into practice. In this context, Katya's proposed fellowship is fully supported and aligns directly with Trust priorities.

GOSH is committed to equality, diversity, and inclusion and to supporting flexible and sustainable clinical academic career pathways. We will ensure that Katya is supported to fully engage with the fellowship while maintaining clinical identity and professional development. Katya is currently employed at Great Ormond Street Hospital as Head of Music Therapy (1.0 WTE), with responsibility for the strategic leadership, management, and clinical delivery of the Music Therapy service. Her role

includes clinical practice, staff management, service development, and contribution to multidisciplinary care within paediatric inpatient settings.

If her NIHR Pre-Doctoral Clinical Academic Fellowship application is successful, GOSH fully supports the required changes to her job plan to enable her to undertake the award on a full-time basis. During the fellowship period, Katya will be released from her substantive clinical and managerial duties to ensure she has protected time to complete the proposed research and training programme in line with NIHR expectations.

To facilitate this, GOSH will put in place appropriate backfill arrangements to cover Katya's clinical and managerial responsibilities. This will involve a formal recruitment process to appoint suitable temporary cover for the Music Therapy service, ensuring continuity of high-quality clinical care and leadership throughout the fellowship period. The Trust is fully committed to supporting this backfill and has experience in facilitating similar arrangements for staff undertaking externally funded research training awards.

These arrangements demonstrate GOSH's clear organisational commitment to supporting Katya's clinical academic development and ensuring that the fellowship is feasible, deliverable, and protected from competing clinical demands.

During the fellowship, Katya will have access to the full range of research infrastructure available at GOSH and UCL GOS ICH, including the NIHR Great Ormond Street Hospital Biomedical Research Centre (NIHR GOSH BRC), the only dedicated paediatric BRC in the UK. First awarded in 2007 and currently funded until November 2027, the NIHR GOSH BRC provides world-leading facilities, methodological expertise, and access to over 200 rare disease populations. This environment offers an excellent platform for high-quality research training and development.

She will also have access to the NIHR GOSH Clinical Research Facility, which has a strong history of patient and public involvement and engagement with children, young people, and families. These resources will support her development in research governance, public involvement, and dissemination, all of which are core competencies for a future independent clinical academic.

Katya's proposed research, exploring the perceived psychosocial, clinical, and experiential impacts of music therapy in paediatric inpatient settings, aligns closely with the strategic priorities of GOSH and UCL GOS ICH. The project addresses a clear gap in the evidence base relating to patient experience, emotional wellbeing, and holistic care in hospitalised children.

Strengthening the evidence base for music therapy is essential to informing service development, supporting commissioning decisions, and underpinning future interventional research within paediatric inpatient services.

The proposed work is appropriately scoped for a pre-doctoral award and represents a strong and coherent foundation for progression to doctoral-level research.

Katya will be supported by an experienced and multidisciplinary supervisory team. Her Primary Academic Supervisor will be Professor Jo Wray, Senior Research Fellow in the Centre for Outcomes and Experience Research in Children's Health, Illness and Disability (ORCHID) at GOSH, Health Psychologist in the Heart and Lung Directorate at GOSH, and Associate Professor at the Institute of Cardiovascular Science, University College London. Professor Wray has a strong track record in supervising early-career clinical academics and has been closely involved in the development of this application. Formal monthly supervision meetings will ensure progress against training and research objectives and alignment with organisational and NIHR priorities.

Additional senior academic support will be provided by Professor Annie Heiderscheit, Professor of Music Therapy and Director of the Cambridge Institute for Music Therapy Research at Anglia Ruskin University, offering discipline-specific expertise in music therapy research.

Katya will remain embedded within ORCHID, which has a clear remit to support Allied Health Professionals in developing high-quality research. She will participate in regular research seminars, journal clubs, and mentoring activities and will have access to senior clinical academic mentorship through the NIHR GOSH BRC, including support from Dr Graeme O'Connor, Biomedical Research Centre Clinical Academic Lead, who previously supported her during an NIHR BRC internship.

Great Ormond Street Hospital for Children
NHS Foundation Trust

GOSH fully supports Katya's longer-term clinical academic trajectory. Subject to successful completion of this fellowship, we are committed to supporting her progression to an NIHR Doctoral Fellowship and to maintaining an appropriate balance between clinical practice and academic development.

As Head of Therapies at GOSH, I confirm that Katya has my full support to undertake this one-year, full-time NIHR Pre-Doctoral Clinical Academic Fellowship, with a planned start date of October 2026. I am confident that the training, supervision, and research environment provided will enable her to successfully complete the fellowship and progress towards becoming an independent clinical academic in paediatric music therapy.

Libby Hough
Head of Therapies
Great Ormond Street Hospital for Children NHS Foundation Trust

Activity - Protected Research and Training Time	Oct-26	Nov-26	Dec-26	Jan-27	Feb-27	Mar-27	Apr-27	May-27	Jun-27	Jul-27	Aug-27	Sep-27
Training & Conferences												
PCAF induction and training needs assessment												
MRes core modules (research design, statistics, qualitative methods)												
MRes dissertation planning												
MRes dissertation writing												
MRes dissertation submission												
Cultural Competence at The Centre for Ethnic Health Research (3 x half days)												
BAMT Conference 2026												
12th Nordic Music Therapy Conference												
Realist Review (theory-driven evidence synthesis)												
Refinement of realist review question and scope												
Exploratory scoping searches (theory building)												
Realist review protocol development												
Search strategy development with GOSH/UCH librarian												
Database and grey literature searches												
Screening for relevance to programme theory												
Full-text screening and theory refinement												
Data extraction (contexts, mechanisms, outcomes)												
Assessment of rigour and relevance (realist principles)												
Realist synthesis and programme theory refinement for NHS inpatient paediatric contexts												
PPIE input on experiential relevance, to inform/refine research question for review												
PPIE feedback on realist synthesis and implications												
Co-produced dissemination outputs												
Realist review manuscript preparation												
Preparation of Doctoral Fellowship application												
Ongoing Activities												
PPI meetings (30 mins - 1 hour)												
Meetings with the supervisory team (monthly)												
Professional supervision (1 hr per month)												
Peer supervision (1hr per month)												